



When to call the doctor, and when to go

The one-page traffic light for the worrying moments. Put it on the fridge.

Emergency departments are very good at fixing specific, fixable problems — and very hard on people with dementia. Loud, bright, disorienting, and a common trigger for delirium. So the goal is not “never go.” **The goal is to go when it counts, and to have a calmer plan for everything else.** The word that decides it is nearly always **how fast**.

RED Call emergency services now

MINUTES TO HOURS

- **Signs of stroke** — face drooping, arm weakness, trouble speaking. Note the time it started.
- **A fall with a hard knock to the head** — especially on blood thinners, even if they seem fine.
- **Chest pain, or real trouble breathing.**
- **A burn, a wound, or an injury that frightens you.**
- **Confusion so sudden and so severe it is nothing like their normal.**

You are not overreacting by calling. *Sudden* is the word that matters.

AMBER Call the doctor's office this week

OVER DAYS

- New or worse confusion — especially if it **comes and goes** through the day.
- A **new medication**, then a new problem.
- Eating or drinking noticeably less.
- Behaviour that is **new for them** — more agitated, more withdrawn, more suspicious.
- Any suspicion of infection: burning urine, a cough, a fever, a wound that looks angry.
- Constipation going on for days, or a fall without injury.

Amber deserves a **phone call, not a waiting room**. Most of these have causes a doctor can sort out without a hospital.

GREEN Write it down, raise it at the next visit

OVER WEEKS TO MONTHS

- Gradually needing more help with dressing, washing, or finding words.
- Sleep or appetite slowly drifting.
- More repeated questions than there used to be.
- Slowly losing interest in things they used to enjoy.

These are **information, not alarms**. Write them down **with a date** so the next appointment is worth having.

The one rule underneath all three

A sudden change is medical until proven otherwise. Not a personality problem, not “the dementia getting worse,” not something to manage with a better technique. Fine last week and not fine this week is the pattern of infection, pain, constipation, a new medication, or a delirium — and every one of those is treatable. **If you are unsure, assume it needs assessing and make the call.**

Before you dial, have these ready

1. Their name, age, and that they have dementia.
2. **What is different, and since when.**
3. Their current medication list.
4. Anything new in the last two weeks.
5. What they are normally like on a good day.

A sentence to borrow

If you are not being heard, say the change out loud, with the timeline:

“This is a sudden change from her baseline. She was independent on Tuesday and she is not making sense today. I am worried about a delirium.”